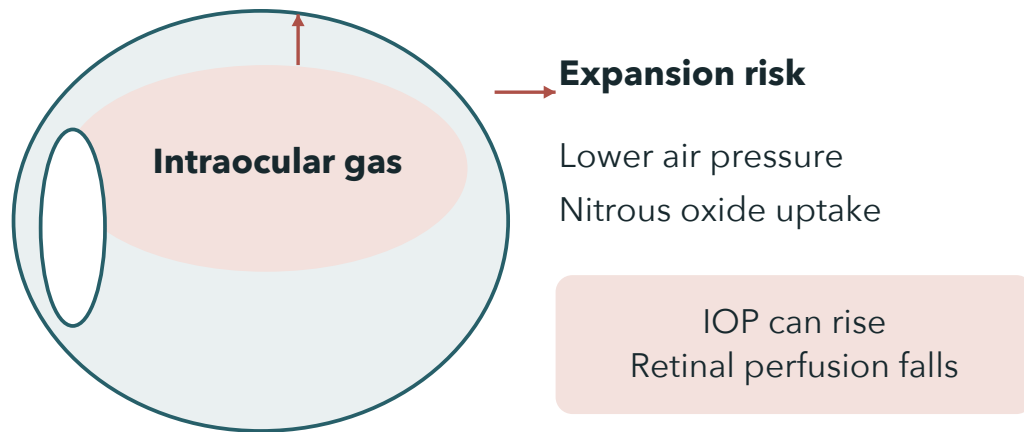


Intraocular gas: safe travel and anaesthesia

Clinical M3-M5 | Focused Type B notes

A bubble changes the safety rules

Expansion can threaten retinal perfusion



Bubble remains: no flying and no nitrous oxide

Original schematic. Lower ambient pressure or nitrous-oxide entry can enlarge intraocular gas, raising IOP and reducing retinal blood flow.

While any gas remains

Exposure	Advice
Flying/high altitude	Avoid until the treating team confirms it is safe.
Nitrous oxide, including gas-and-air analgesia	Avoid; warn emergency and anaesthetic teams.
Required head positioning	Follow the individual retinal surgeon's instructions.

Normal IOP is not clearance to fly

A normal pressure measurement before travel does not prevent expansion during the exposure. Pressure-lowering drops and head positioning do not replace the gas precautions.

References RCOphth: Intraocular gas safety alert | ASRS: Macular hole

Make the postoperative plan usable

Before leaving the retina service

- 1 Know what was placed in the eye and whether a bubble remains.
- 2 Carry a clear gas warning and tell any clinician before analgesia or anaesthesia.
- 3 Confirm the expected review and the surgeon's criteria for lifting travel restrictions.

Safety-net symptoms

Change	Action
Increasing pain or sudden visual decline	Urgent contact with the retinal/eye emergency team.
New field curtain or many new floaters	Urgent retinal assessment.
Difficulty following positioning	Contact the surgical team for an individual plan.

Do not guess a universal date

Gas duration varies with agent, concentration and the eye. Base clearance on the treating team's advice and confirmed gas status, not a fixed interval remembered from another patient.